Processed At :
Plot No 436C, 3rd Floor, Industrial
Plot No 436, Sector-82, JLPL, SAS
Nagar, Mohali, Punjab.

Polo Labs
Right Treatment begins with Right Diagnosis

Processed At: Plot No.436C, 3rd Floor, Industrial Plot No.436, Sector-82, JLPL, SAS Nagar, Mohali, Punjab.

REPORT

Patient Name	: **DEEPAK BANSAL (53Y/M)**	Tests Done	: **ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE,FBS**
Referred By	: **SELF**		
Address	: **(1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001**		

Report Availability Summary

Note: Please refer to the table below for status of your tests.

7 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready

Processed At :
Plot No 436C, 3rd Floor, Industrial
Plot No 436, Sector-82, JLPL, SAS
Nagar, Mohali, Punjab.

Polo Labs
Right Treatment begins with Right Diagnosis

Processed At: Plot No.436C, 3rd Floor, Industrial Plot No.436, Sector-82, JLPL, SAS Nagar, Mohali, Punjab.

REPORT

Patient Name	: **DEEPAK BANSAL (53Y/M)**	Tests Done	: **ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE,FBS**
Referred By	: **SELF**		
Address	: **(1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001**		

Tests Outside Reference Range

Note: Please refer to the table below for tests outside reference range.

Test Name	Observed Value	Units	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTES - ABSOLUTE COUNT	**3.01**	X 10^3 / µL	1.0-3.0
NEUTROPHILS - ABSOLUTE COUNT	**7.97**	X 10^3 / µL	2.0-7.0
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**14.3**	%	11.6-14
TOTAL LEUCOCYTES COUNT (WBC)	**12.38**	X 10^3 / µL	4.0 - 10.0
DIABETES			
AVERAGE BLOOD GLUCOSE (ABG)	**318**	mg/dL	90-120
FASTING BLOOD SUGAR(GLUCOSE)	**275.71**	mg/dL	70-100
HbA1c	**12.7**	%	< 5.7
LIPID			
HDL / LDL RATIO	**0.2**	Ratio	> 0.40
HDL CHOLESTEROL - DIRECT	**35**	mg/dL	40-60
LDL / HDL RATIO	**5.1**	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	**181.68**	mg/dL	< 100
NON-HDL CHOLESTEROL	**208.23**	mg/dL	< 160
TC/ HDL CHOLESTEROL RATIO	**6.9**	Ratio	3 - 5
TOTAL CHOLESTEROL	**244**	mg/dL	< 200
TRIG / HDL RATIO	**6.72**	Ratio	< 3.12
TRIGLYCERIDES	**238**	mg/dL	< 150
VLDL CHOLESTEROL	**47.59**	mg/dL	5 - 40
RENAL			
CREATININE - SERUM	**0.56**	mg/dL	0.72-1.18

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

Processed At: Plot No.436C, 3rd Floor, Industrial Plot No.436, Sector-82, JLPL, SAS Nagar, Mohali, Punjab.

REPORT

Patient Name	: DEEPAK BANSAL (53Y/M)	**Sample Collected on (SCT)**	: 02 May 2026 14:35
Referred By	: SELF	**Sample Received on (SRT)**	: 02 May 2026 15:01
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: 2 May 2026 15:38
		Sample Type | Barcode	: FLUORIDE PLASMA | FG148058

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	**PHOTOMETRY**	**275.71**	**mg/dL**

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-POD METHOD

Tests Done : BLOOD SUGAR (F)

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

Scan QR to verify(valid for 30 days from release time)

REPORT

Patient Name	: DEEPAK BANSAL (53Y/M)	Sample Collected on (SCT)	: 02 May 2026 14:35
Referred By	: SELF	Sample Received on (SRT)	: 02 May 2026 15:01
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	Report Released on (RRT)	: 2 May 2026 17:00
		Sample Type | Barcode	: SERUM | FT508911

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
TOTAL CHOLESTEROL	PHOTOMETRY	244	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	35	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	181.68	mg/dL	< 100
TRIGLYCERIDES	PHOTOMETRY	238	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	6.9	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	6.72	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	5.1	Ratio	1.5-3.5
HDL / LDL RATIO	CALCULATED	0.2	Ratio	> 0.40
NON-HDL CHOLESTEROL	CALCULATED	208.23	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	47.59	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Tests Done : ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

REPORT

Patient Name	: **DEEPAK BANSAL (53Y/M)**	**Sample Collected on (SCT)**	: **02 May 2026 14:35**
Referred By	: **SELF**	**Sample Received on (SRT)**	: **02 May 2026 15:01**
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: **2 May 2026 17:00**
		Sample Type | Barcode	: **SERUM | FT508911**

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
ALKALINE PHOSPHATASE	PHOTOMETRY	104.66	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.76	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.12	mg/dL	0 - 0.20
BILIRUBIN (INDIRECT)	CALCULATED	0.64	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	24.13	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	20.92	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	22.64	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	0.92	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.33	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.1	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.23	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.27	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Diazonium salt DPD method
BILD - Diazonium salt DPD method
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg[1]method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Tests Done : ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

REPORT

Patient Name	: **DEEPAK BANSAL (53Y/M)**	**Sample Collected on (SCT)**	: **02 May 2026 14:35**
Referred By	: **SELF**	**Sample Received on (SRT)**	: **02 May 2026 15:01**
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: **2 May 2026 17:00**
		Sample Type | Barcode	: **SERUM | FT508911**

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	11.86	mg/dL	7.94 - 20.07
CREATININE - SERUM	**PHOTOMETRY**	**0.56**	**mg/dL**	**0.72-1.18**
BUN / Sr.CREATININE RATIO	CALCULATED	21.18	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	25.38	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	45.32	Ratio	< 52
CALCIUM	PHOTOMETRY	9.98	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	4.8	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Tests Done : ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

Processed At: Plot No.436C, 3rd Floor, Industrial Plot No.436, Sector-82, JLPL, SAS Nagar, Mohali, Punjab.

REPORT

Patient Name	: **DEEPAK BANSAL (53Y/M)**	**Sample Collected on (SCT)**	: **02 May 2026 14:35**
Referred By	: **SELF**	**Sample Received on (SRT)**	: **02 May 2026 15:01**
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: **2 May 2026 17:00**
		Sample Type | Barcode	: **SERUM | FT508911**

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	123	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	9.14	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	3.78	µIU/mL	0.54-5.30

Comments : SUGGESTING THYRONORMALCY

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Tests Done : ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

REPORT

Patient Name	: DEEPAK BANSAL (53Y/M)	**Sample Collected on (SCT)**	: 02 May 2026 14:35
Referred By	: SELF	**Sample Received on (SRT)**	: 02 May 2026 15:01
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: 2 May 2026 17:00
		Sample Type | Barcode	: SERUM | FT508911

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	118	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease
<15 : Kidney Failure

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

Tests Done : ESSENTIAL HEALTH CHECKUP PROFILE - ALL IN ONE

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

REPORT

Patient Name	: DEEPAK BANSAL (53Y/M)	**Sample Collected on (SCT)**	: 02 May 2026 14:35
Referred By	: SELF	**Sample Received on (SRT)**	: 02 May 2026 15:15
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: 2 May 2026 17:28
		Sample Type | Barcode	: EDTA Whole Blood | FT508912

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	**H.P.L.C**	**12.7**	**%**

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	**CALCULATED**	**318**	**mg/dL**

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

REPORT

Patient Name	: DEEPAK BANSAL (53Y/M)	**Sample Collected on (SCT)**	: 02 May 2026 14:35
Referred By	: SELF	**Sample Received on (SRT)**	: 02 May 2026 15:15
Address	: (1330014263),POLO LABS AMBALA,5358/6, GROUND FLOOR, GEETA GOPAL CHOWK, AMBALA CANTT, HARYANA - 133001,133001	**Report Released on (RRT)**	: 2 May 2026 17:28
		Sample Type | Barcode	: EDTA Whole Blood | FT508912

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	14.7	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	46.3	%	40.0-50.0
Total RBC	HF & EI	5.26	X 10^6/µL	4.5-5.5
Mean Corpuscular Volume (MCV)	Calculated	88	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	27.9	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	31.7	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	45.7	fL	39-46
Red Cell Distribution Width (RDW - CV)	**Calculated**	**14.3**	**%**	**11.6-14**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	239.2	-	*Refer Note below
MENTZER INDEX	Calculated	16.7	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	**HF & FC**	**12.38**	**X 10^3 / µL**	**4.0 - 10.0**
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	64.4	%	40-80
Lymphocytes Percentage	Flow Cytometry	24.3	%	20-40
Monocytes Percentage	Flow Cytometry	7.3	%	2-10
Eosinophils Percentage	Flow Cytometry	3.2	%	1-6
Basophils Percentage	Flow Cytometry	0.6	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	**Calculated**	**7.97**	**X 10^3 / µL**	**2.0-7.0**
Lymphocytes - Absolute Count	**Calculated**	**3.01**	**X 10^3 / µL**	**1.0-3.0**
Monocytes - Absolute Count	Calculated	0.9	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.07	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.4	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	297	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	11.6	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	13.9	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	36.4	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.35	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. WBCs:Mild Leukocytosis is present. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

~~ End of report ~~

Tests Done : HBA PROFILE,HEMOGRAM

Report Remarks : Clinically Tested by: Thyrocare Technologies Ltd

Dr Vishal Salhotra
MD(Path)

Scan QR to verify(valid for 30 days from release time)

CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**